

Patient Name	: DR ARCHANA JAIN	Registered On	: 31/05/2025 05:44PM
Patient Id	: 40102556346	Sample Collected On	: 31/05/2025 03:08PM
Age/DOB/Gender	: 57Y /0M /0Days/ {FEMALE}	Reported On	: 31/05/2025 06:49PM
Nationality	: INDIAN	Sample UID No.	: P011B000364794
Customer Type	: B2B - Lab	Customer Name	: DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	: SELF		

Glycosylated Haemoglobin (HbA1c)

<u>Investigation</u>	<u>Result</u>	<u>Units</u>	<u>Biological Reference Interval</u>
Glycosylated Haemoglobin(HBA1C)	7.8	%	Non-diabetic adults >=18 years <5.7 At risk (Pre-diabetes) 5.7-6.4 Diagnosing Diabetes >=6.5 Therapeutic goals for glycemic control Age > 19 years: < 7 For Age < 19 year: < 7.5

Mean Blood Glucose 177.16 mg/dl
Sample Type : EDTA Whole Blood.
Method : High-performance liquid chromatography.

Interpretations:

Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate. Comments: HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations Disclaimer: The above result relate only to the specimens received and tested in laboratory and should be always correlate with clinical findings and other laboratory markers. Improper specimen collection, handling, storage and transportation may result in false negative/Positive results.

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Dr DIPANJAN BISWAS
MBBS,DCP CONSULT PATHOLOGIST

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Age/DOB/Gender	: 57Y /0M /0Days/ {FEMALE}	Reported On	: 31/05/2025 06:58PM
Nationality	: INDIAN	Sample UID No.	: P011B000364794
Customer Type	: B2B - Lab	Customer Name	: DR. BR AMBEDKAR CLINICAL LAB RAIPUR
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Ultra Thyroid Stimulating Hormone (TSH)

<u>Investigation</u>	<u>Result</u>	<u>Units</u>	<u>Biological Reference Interval</u>
ULTRA Thyroid Stimulating Hormone (TSH)	4.63	uIU/mL	0.27 - 4.2 Frist trimester: 0.3- 4.5 Second trimester : 0.5 -4.6 Third trimester : 0.8 -5.2

- Sample Type : Serum.
- Method : ECLIA.

- Comments :
 - TSH levels are subject to circadian variation, reaching peak levels between 2 - 4 a.m. and at a minimum between 6-10 pm .
 - The variation is of the order of 50% , hence time of the day has influence on the measured serum TSH concentrations .

- Clinical Use :
 - Diagnose Hypothyroidism and Hyperthyroidism.
 - Monitor T4 replacement or T4 suppressive therapy.
 - Quantify TSH levels in the subnormal range.

- Increased Levels : Primary hypothyroidism, Subclinical hypothyroidism, TSH dependent Hyperthyroidism Thyroid hormone resistance .
- Decreased Levels : Graves disease, Autonomous thyroid hormone secretion, TSH deficiency.

- References :
 - Henry's Clinical Diagnosis and Management, 23rd edition.
 - Tietz Fundamentals of Clinical Chemistry and Molecular Diagnosis , 7th edition.

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MD PATHOLOGY

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Vitamin B12 (Cyanocobalamin)

<u>Investigation</u>	<u>Result</u>	<u>Units</u>	<u>Biological Reference Interval</u>
Vitamin B12 (Cyanocobalamin)	423	pg/mL	191 - 663

- Sample type : Serum.
- Method: ECLIA.

• Comments : Vitamin B12 performs many important functions in the body, but the most significant function is to act as coenzyme for reducing ribonucleotides to deoxyribonucleotides, a step in the formation of genes. Inadequate dietary intake is not the commonest cause for cobalamine deficiency. The most common cause is malabsorption either due to atrophy of gastric mucosa or diseases of terminal ileum. Cobalamine deficiency leads to Megaloblastic anemia and demyelination of large nerve fibres of spinal cord. Normal body stores are sufficient to last for 3-6 years. Sources of Vitamin B12 are liver, shellfish, fish, meat, eggs, milk, cheese & yogurt.

• Decreased Levels :

1. Lack of Intrinsic factor: Total or partial gastrectomy, Atrophic gastritis, Intrinsic factor antibodies.
2. Malabsorption: Regional ileitis, resected bowel, Tropical Sprue, Celiac disease, pancreatic insufficiency, bacterial overgrowth & achlorhydria.
3. Loss of ingested vitamin B12: fish tapeworm.
4. Dietary deficiency: Vegetarians.
5. Congenital disorders: Orotic aciduria & transcobalamine deficiency.
6. Increased demand: Pregnancy specially last trimester.

• Increased Levels :

Chronic renal failure, Congestive heart failure, Acute & Chronic Myeloid Leukemia, Polycythemia vera, Carcinomas with liver metastasis, Liver disease, Drug induced cholestasis & Protein malnutrition

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Customer Type	: B2B - Lab	Customer Name	: DR. BR AMBEDKAR CLINICAL LAB RAIPUR
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Vitamin D Total - 25 Hydroxy

<u>Investigation</u>	<u>Result</u>	<u>Units</u>	<u>Biological Reference Interval</u>
25-OH Vitamin D	16.44	ng/mL	Deficiency-<20 Insufficiency-20-<30 Sufficiency-30-100

• Sample type : Serum.

• Method : ECLIA.

Interpretation :

Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol (vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.

Testing for 25(OH)vitamin D is recommended as it is the best indicator of vitamin D nutritional status as obtained from sunlight exposure & dietary intake. For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.

During monitoring of oral vitamin D therapy- suggested testing of serum 25(OH)vitamin D is after 12 weeks or 3 mths of treatment. However, the required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal (especially winter) & individual variability depending on age, body fat, sun exposure, physical activity ,genetic factors(especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism Vitamin D toxicity is known but very rare.

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Nationality	: INDIAN	Sample UID No.	: P011B000364794
Customer Type	: B2B - Lab	Customer Name	: DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	: SELF		

Free T3

<u>Investigation</u>	<u>Result</u>	<u>Units</u>	<u>Biological Reference Interval</u>
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Free T3	2.76	pg/mL	2.0 - 4.4
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• Sample Type : Serum.

• Method : ECLIA.

• Clinical Use :

Diagnose and monitor treatment of Hyperthyroidism.

Clarify thyroid status in presence of possible protein binding abnormality.

• Increased Levels: Graves disease, T3 thyrotoxicosis, Thyroid hormone resistance, Functional thyroid adenoma (T3 producing).

• Decreased Levels: Nonthyroidal illness, Hypothyroidism , Nutritional deficiency.

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Free T4

<u>Investigation</u>	<u>Result</u>	<u>Units</u>	<u>Biological Reference Interval</u>
Free T4	1.33	ng/dl	0.93 - 1.7
• Sample Type : Serum. • Method : ECLIA. • Clinical Use : 1. Initial test of thyroid function in patients with suspected thyroid dysfunction. 2. Assess thyroid status in patients with abnormal total T 4 concentrations. 3. Distinguish Euthyroid hyperthyroxinemias from hypothyroidism.			

- Increased Levels: Thyroid hormone resistance, Hyperthyroidism.
- Decreased Levels: Primary hypothyroidism, Secondary hypothyroidism.

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